Medical Records - CONFIDENTIAL

FROM: TX - Texas Colon and Rectal Surgeons

Cindy G
4207 JAMES CASEY ST STE 201 BLDG A, AUSTIN, TX 78745-1193
Phone: (512) 447-8911
Fax: (512) 447-8761

TO:

Name: DESCROIX, RICHARD

DOB: 03/21/1956

Date Range: 01/01/2026 to 03/03/2026

This document contains the following records of the patient:
- **Clinical Documents**

Note: RICHARD DESCROIX

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DESCROIX, Richard (id #872031, dob: 03/21/1956)

Clinical Documents

DESCROIX, Richard (id #872031, dob: 03/21/1956)

click here to view original file

PAM HEALTH AT HOME

Fax

To: SOLIS RICARDO
From: Pam Health at Home - Texas
Fax: 18554271214
Pages: 3
Phone:
Date: 02-27-2026 6:13 PM
Re: Order(s) for Signature
Comments:

This fax contains orders for signature.
Please have the physician/clinician noted on this order.
Please sign, date, and return all pages to 1-512-295-7070.
Thank you!

PAM Health at Home – Texas
825 Main Street, Suite 110, Buda, TX 78610
Ph: 512-295-7000; Fax: 512-295-7070

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Order No:485-26126

HOME HEALTH CERTIFICATION AND PLAN OF CARE

1. Patient's HI Claim No.	2. Start Of Care Date	3. Certification Period	4. Medical Record No.	5. Provider No.
9AT0PQ6UH38	10/27/2025	02/24/2026 - 04/24/2026	7702	747688

6. Patient's Name and Address
Descroix, Richard (7702)
456 Belony Loop
Buda, TX 78610
Phone: (760) 750-2521

7. Provider's Name, Address and Telephone Number
PAM Health at Home
825 Main St Ste 110
Buda, TX 78610-3273
Phone: (512) 295-7000, Fax: (512) 295-7070

8. Date of Birth: 03/21/1956 | 9. Sex: ☒ M ☐ F

10. Medications: Dose/Frequency/Route (N)ew (C)hanged

Medication	Dose	Frequency	Route	Status
Acetaminophen 500 mg capsule [Amount Of/ Dosage Form: 0 Capsule]	1-2	every 4-6 hours as needed	oral	LS - PRN/As Needed
Advil 200 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	1-2	2-3 times a day as needed	oral	LS - PRN/As Needed
Ferrous sulfate 325 mg (65 mg iron) tablet [Amount Of/ Dosage Form: 1 Tablet]	325mg	daily	oral	LS
Lidocaine 4 % topical patch [Amount Of/ Dosage Form: 0 Patch]	1	daily as needed	topical	LS - PRN/As Needed
Probiotic 10 billion cell capsule [Amount Of/ Dosage Form: 0 Capsule]	10 billion	daily	oral	LS
Tramadol 50 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	1	every 6 hrs as needed	oral	LS - PRN/As Needed
Vitamin D2 20 mcg-vitamin K1 120 mcg/4 drops oral [Amount Of/ Dosage Form: 0 Metric Drop]	120mcg	daily	oral	LS

11. ICD-9 / 10 CM Principal Diagnosis

Code	Code Description	Date	O/E
Z48.3	Aftercare following surgery for neoplasm		

12. ICD-9 / 10 CM Surgical Procedure

Code	Code Description	Date	O/E

13. ICD-9 / 10 CM Other Pertinent Diagnosis

Code	Code Description	Date	O/E
C20	Malignant neoplasm of rectum		
K63.1	Perforation of intestine (nontraumatic)		
Z43.3	Encounter for attention to colostomy		
G72.89	Other specified myopathies		

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Z45.2	Encounter for adjustment and management of vascular access device
D64.9	Anemia, unspecified
E87.0	Hyperosmolality and hypernatremia
E87.6	Hypokalemia
N40.0	Benign prostatic hyperplasia without lower urinary tract symptoms
Z79.891	Long term (current) use of opiate analgesic

14. DME and Supplies:
DME: Walker, Wheel Chair
ostomy supplies, ALCOHOL WIPES, NS GLOVES, PROBE COVERS, PPE MASK

15. Safety Measures:
911 Emergency / Protocol, Assistance with IADLs, Emergency Plan / Measures, Emergency Plan Developed, Equipment Safety, Fall / Safety Precautions, Hazard-Free Environment, Infection Control Standard Measures and Precautions, Keep Pathways Clear and Safe, Medication Safety / Storage, Proper Hazardous Waste Disposal, Safety in ADLs Skin Care, Slow Position Change, Support During Transfer and Ambulation, Use of Assistive Devices, Walker / Cane

16. Nutritional Req.
Protein Hi Diet, Regular.

17. Allergies:
No Known Allergies

18.A. Functional Limitations
☐ 1- Amputation ☐ 4- Hearing ☑ 7- Ambulation
☑ 2- Bowel/Bladder (incontinence) ☐ 5- Paralysis ☐ 8- Speech
☐ 3- Contracture ☑ 6- Endurance ☐ 9 - Legally Blind
☐ A- Dyspnea with minimal exertion
☑ B- Other (Specify): pain

18.B. Activities Permitted
☐ 1- Complete Bedrest ☑ 5- Exercises Prescribed ☐ 9- Cane
☐ 2- Bedrest BRP ☐ 6- Partial Weight Bearing ☐ A- Wheelchair
☐ 3- Up as Tolerated ☑ 7- Independent at Home ☐ B- Walker
☐ 4- Transfer Bed/Chair ☐ 8- Crutches ☐ C- No Restrictions
☑ D- Other (Specify):SBA

19. Mental Status
☑ 1 - Oriented ☐ 2 - Comatose ☐ 3 - Forgetful ☐ 4 - Depressed ☐ 5 - Disoriented ☐ 6 - Lethargic ☐ 7 - Agitated
☐ 8 - Other (Specify):

20. Prognosis: ☐ 1- Poor ☐ 2- Guarded ☐ 3- Fair ☑ 4- Good ☐ 5- Excellent ☐ 6- Other (Specify)

21. Treatments and Orders for Disciplines:
Frequency

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8. Date of Birth: 03/21/1956 | 9. Sex: ☑ M ☐ F

SN Frequency: 1W4, 1EOW4 Starting Week of 02/24/2026,SN PRN Visits/Hours: 3 (OSTOMY complications)

Vitals Normal Range

BP Systolic: 90 - 160
BP Diastolic: 56 - 100
Pulse: 56 - 100
Respiratory: 12 - 28
Temperature: 96.5 - 100.5
O2 Saturation: 90 - 100

Vitals Physician Alert Range

Temperature: < 96.5 or > 100.5
Pulse: < 56 or > 100
Respiratory: < 12 or > 28
BP Systolic: < 90 or > 160
BP Diastolic: < 56 or > 100
O2 Saturation: < 90
Pain: > 7

SN

BEST PRACTICE FALL PREVENTION:

• Clinician will perform fall assessment and instruct patient /caregiver on fall prevention.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION:

• Clinician to assess/instruct on high risk and other medications to include effects, potential adverse effects, interactions and duplicate therapy, dose route and frequency.

BEST PRACTICE MONITOR AND MITIGATE PAIN:

• Clinician will instruct patient /caregiver on methods to mitigate pain to include medications and non-pharmacological interventions to decrease pain.

GENITOURINARY:

• Clinician to obtain Urine for U/A and C& S as needed for s/s of UTI via clean catch and notify physician of results

OSTOMIES:

• Clinician to assess/Perform/instruct patient/caregiver on colostomy care using aseptic technique as follows:
Dressing changes 2x weekly and PRN
Peristoma - 3 O'clock: Cleanse with normal saline, blot dry. Apply Sorbact, apply light pack to wound - ribbon. Include site within ostomy wafer, change 2x weekly and PRN.
Stoma: Remove appliance with adhesive remover. Cleanse with water. Apply barrier film. Apply stoma ring. Apply pouch: - Hollister 2pc flat appliance. Empty pouch when 1/3 to 1/2 full. Change pouch if experiencing any burning or itching. Stoma barrier to be cut 1/8" larger than stoma.

• SN to instruct patient/caregiver on ostomy management as follows: Showering and bathing will not hurt your ostomy or the appliance. Some people prefer to shower without their appliance on the days they change their pouching system. You can decide what you prefer. If you use soap make sure it does not contain any oils or lotions as this can interfere with how securely your faceplate adheres to your skin. Be sure to rinse the skin well. The residue from the soap could prevent the faceplate from sticking as well. Dry your skin gently but well before applying the new faceplate.

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8. Date of Birth: 03/21/1956 **9. Sex:** ☒ M ☐ F

• SN to instruct patient/caregiver on ostomy management as follows: Change your pouch every 4-7 days.

• SN to instruct patient/caregiver on ostomy management as follows: Eat 5-6 small meals a day. Ensure your diet is well balanced. Avoid skipping meals since this may cause diarrhea and gas. Introduce one new food at a time to see the effect on your body. A food journal may be helpful. Chew all foods well. Cook foods well until they are tender. Be mindful of foods that may increase gas, odor, stoma obstruction, diarrhea, or constipation. When the colon is removed to create an ileostomy there is a greater risk for dehydration and electrolyte imbalance. This is because the colon usually absorbs the majority of the fluids and electrolytes the body needs to function. Diarrhea, vomiting, or perspiration can increase the risk of dehydration and electrolyte imbalances. To prevent fluid and electrolyte imbalances drink 8-12 cups of fluid per day. Avoid using straws(will increase gas). Drink caffeinated beverages moderately as they can lead to dehydration.

• SN to instruct patient/caregiver on ostomy management as follows: Whenever possible, stand when applying your ostomy appliance. This will help keep the skin around your ostomy smooth and flat, and will improve the quality of the seal you get between your faceplate and the skin.

HOSPITALIZATION RISK FACTORS:

• Clinician will instruct the patient and/or caregivers on the following unstable disease processes that may lead to unplanned hospitalizations: perforated bowel s/p resection with new colostomy, septic shock, colorectal cancer, anemia.

• Clinician will instruct patient/caregiver on ways to prevent unintentional weight loss and poor nutrition (high calorie foods, recording (wt/diet), etc

• Clinician will instruct patient and/or caregivers on polypharmacy, duplicate medications, adverse effects, et and compliance/understanding of medication regimen.

• Clinician to instruct patient and/or caregivers on ways to avoid exhaustion and fatigue to include adequate sleep, nutrition, exercise, etc.

• Clinician will instruct patient and/or caregivers on fall prevention to include clear pathways, adequate lighting, assistive devices, slow change in position, etc.

60 Day Recert Summary

Physician: Oscar Rios MD
Date: 02/19/2026
Vital Sign Range:
Temperature: 97 - 98.7
Pulse: 67 - 95
Respiration: 16 - 18
Blood Pressure
Systolic: 98 - 128
Diastolic: 56 - 84
O2 Saturation: 95 - 100
Labs: n/a

Care Summary: 69-year-old male with PMH of BPH, and right inguinal hernia repair on 9/24. Pt has recent Dx Colorectal CA and had resection of proximal ilium and creation of colostomy in October 2025. He was admitted to PAM IPR and PAM health at home post op. He had surgical removal of rectal tumor and placement of Port a cath Lt upper chest on 1/20/26 and had first dose chemotherapy on 2/16/26. Pt is alert and oriented, pleasant and cooperative. He C/O significant fatigue and is having back, pelvic and rectal pain 5/10 dull. Lungs are clear and HR regular, no edema present, Pt has incision Lt upper chest which is closed and healing well with no signs of infection. Pt stated he saw Dr. Solis on 2/17 and MD placed cameral in rectal cavity and said the incision is healing well. Pt stated he is continuing to have drainage from the rectum which at times is

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serosanguinous or gray in color. Pt has appliance to stoma LL lower abcomen and now is independent with changing the appliance. Noted that he has hernia surrounding the ostomy where abdomen is distended and soft. Stoma appears healthy and pink visible through appliance. He stated Dr Solis said it would be OK to apply lidocaine ointment to the rectal area for pain control and the pt is using a doughnut type cushion to sit on. He is no longer taking oral pain medication. Pt ambulatory in home without device but has walker and cane for community. He lives with his wife in SS home. Pt is homebound due to weakness, fatigue and pain. He requires assistance and must manage colostomy making it very taxing to leave home. Pt stated noted some SOB this am when he first got up and has been having some positional dizziness. Noted that last lab draw, Hg 8.8 and pt stated he received iron infusion at oncology office. Pt is homebound as he requires assistance and it is very taxing to leave home due to weakness, fatigue and pain related to rectal cancer, chemo and need for management of colostomy. Pt continues to need SN for assessment and teaching related to chemotherapy related to colorectal cancer, colostomy care, diet and nutrition, skin integrity, pain management, medication management, fall prevention and safety. Pt is planning on returning to his job which requires the pt to be on the phone and will start in about 6 weeks working from home.

Frequency may be completed with both face to face visits and/or telehealth (if appropriate to meet patient treatment needs).
All qualified disciplines may do non-billable visits to supervise staff and collect oasis data as needed.
Within 5 days of the end of episode, a qualified professional may have a PRN visit to complete a comprehensive/OASIS assessment to establish the need for continuing home health services or discharge.
May accept orders from all physicians caring for the patient.
Any qualified disciplines may perform visit for resumption of care when patient discharged home from inpatient facility.
May resume all care prior to the hospitalization.
PAM Health at Home may discharge patient if remains in facility when certification period expires.
All skilled disciplines may check oxygen saturation pm sob.

I certify in my estimation that the patient will need skilled care from home health for the next 60 days
I attest to the following:
1. RICHARD DESCROIX is under my care.
2. I have received the following patient records from Be Healthy at Home
a. Patient Summary Form
b. Hospital/LTC stay information (if applicable)
c. Home Health Plan of Care
d. And the signature below is my signature
3. Each of these patient records have been incorporated into my patient record.
4. These patients records were considered in advance of my certification of the patient's eligibility for home health services under Medicare.

Risk of Hospitalization

Risks: Unintentional weight loss of a total of 10 pounds or more in past 12 months; Multiple hospitalizations (2 or more) in the past 6 months; Multiple emergency department visits (2 or more) in the past 6 months; Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months; Currently taking 5 or more medications; Currently reports exhaustion;
Interventions: Referrals - SN; Medication Management - Reconciliation, Assess patients: Knowledge,Ability,Resources & Adherence, Education; Enrollment into a Disease Management program - ostomy support group; Patient/Family Education; Fall Prevention Program; Individualized Patient Emergency Care Plan; Care Coordination (Physicians, Hospitals, Nursing homes...)
Risk Factors: Help with managing Medications needed (M2020); More than 2 Secondary Diagnoses (M1022 & M1024); ADL

Order No:485-26125

HOME HEALTH CERTIFICATION AND PLAN OF CARE

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9ATCPQ8UH08	10/27/2025	02/24/2026 - 04/24/2026	7702	747888

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456 Belony Loop
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8. Date of Birth: 03/21/1956 **9. Sex:** ☐ M ☐ F

Assistance Needed (M2102 & M2110) : Home Safety Risks

Psychosocial Assessment

No problem found

Advance Directive

Living will; Healthcare representative
Patient's Advance Directive - Living Will, Durable Power of Attorney for Health Care (Name: Shoshanna Salsman, Phone: (818) 621-9636, Relation: daughter);
Have an Advanced Care Plan and it is documented in the medical record: Yes - Medical Treatment Preferences;
Have a Surrogate Decision Maker and information documented in the medical record: Yes;

Homebound Status

Patient Homebound Reasons
- Is the Patient Homebound: Yes
- Homebound Reasons: Residual weakness; Unable to safely leave home unassisted;
- Patient Chronic and Stable: Yes
- Patient leaving the home require Considerable and Taxing effort : Yes
- Comments: Pt requires assistance and it is very taxing to leave home due to weakness and pain related to rectal cancer, chemo and presence of colostomy.

22. Rehabilitation Potential/Discharge Plans/Goals :

Recertification of Patient Eligibility

I estimate the duration of continued Home Health services for this patient to be
60 days

Rehab Potential

Good to achieve stated goals with skilled interventions and patient's compliance with plan of care.

Discharge Plan

Discharge when medical condition is stable and patient is no longer in need of skilled services
Discharge when goals met

Goals

SN

BEST PRACTICE FALL PREVENTION: - LTG
- LTG-Patient/caregiver will verbalize understanding of interventions to prevent falls by end of episode.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION: - LTG
- LTG-Patient /caregiver will be able to name s/s of adverse side effects to report to MD by end of episode.

BEST PRACTICE MONITOR AND MITIGATE PAIN: - LTG
- LTG-Patient/caregiver will demonstrate decreased pain with ambulation as evidenced by increased activity tolerance by end of episode.

GENITOURINARY: - LTG
- LTG-Patient will tolerate collection of urine without pain and discomfort for S/Sx UTI by end of episode.

Order No:435-28126

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OSTOMIES: - LTG
- LTG-Patient's colostomy will remain free of infection/breakdown by end of episode.

OSTOMIES: - STG
- STG-Patient/caregiver will demonstrate ability to perform ostomy care independently as ordered by 4 weeks.

HOSPITALIZATION RISK FACTORS: - LTG
- Patient and/or caregivers will verbalize understanding of ways to mitigate risk of return to the hospital and unplanned ER use.

23. Clinician's Signature and Date of Verbal SOC Where Applicable: (Electronically Signed)
02/19/2026
Signed By: Symes, Sarah - RN on
02/19/2026 08:20 AM, Cosigned by
Strait, Donna - RN

25. Date HHA Received Signed POT:

24. Physician's Name and Address
RICARDO SOLIS MD
4007 James Casey Drive Ste D150
AUSTIN, TX 78745,NPI: 1891802732, License#: L1757,Phone: (512) 447-6011, Fax: (855) 427-1214

3/2/26

26. I certify/ recertify that this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy or continues to need occupational therapy. This patient is under my care, and I have authorized the services on this plan of care and I or another physician will periodically review this plan. I attest that a valid face-to-face encounter occurred (or will occur) within timeframe requirements and it is related to the primary reason the patient requires home health service

27. Attending Physician's Signature and Date Signed:

28. Anyone who misrepresents,falsifies, or conceals essential information required for payment of Federal funds may be subject to fine, imprisonment, or civil penalty under applicable Federal laws.